

# SPARTAN COACHING

HOSPICE SALES EXCELLENCE

## Medicare/Medicaid Hospice Regulations & Compliance Guide

### MEDICARE HOSPICE ELIGIBILITY (The Gatekeeper Rules)

A patient must meet ALL of these criteria to be Medicare-eligible for hospice:

**1. Medicare Part A Coverage**

Patient must have active Medicare Part A (hospital insurance). If only on Part B, not eligible.

**2. Physician Certification**

A licensed physician MUST certify the patient has a terminal illness with 6-month prognosis or less.

**3. Informed Consent**

Patient (or authorized representative) must sign the Medicare Hospice Election Form (CMS-1525-02).

**4. Prognosis Certainty**

The 6-month window assumes "if disease runs normal course"—patient must clearly be declining.

### COMMONLY ELIGIBLE DIAGNOSES

- **Cancer** Metastatic with systemic symptoms, declining functional status
- **COPD** FEV1 <25% predicted OR resting hypoxemia OR hypercapnia
- **Heart Failure** Class IV (symptomatic at rest), ejection fraction <20%, recent hospitalizations
- **Renal Disease** Serum creatinine >2.5 or GFR <25 (NOT on dialysis)
- **Liver Disease** Bilirubin >2.5 or INR >1.5 OR ascites present
- **Dementia** Feeding tube required, unable to communicate, recurrent infections
- **ALS** Declining respiratory function, difficulty swallowing, weight loss >10%
- **Stroke** Unable to swallow, severe neurological decline, unable to ambulate

### THE 4-STEP REFERRAL PROCESS

**STEP 1: DISCOVERY**

Patient meets eligibility criteria. Care team identifies patient as appropriate for hospice consideration.

**STEP 2: PHYSICIAN EVALUATION**

Primary care physician evaluates patient, confirms terminal diagnosis and 6-month prognosis. No referral without this.

**STEP 3: PATIENT/FAMILY CONSENT**

Physician discusses hospice with patient/family. Patient/representative signs CMS-1525 election form. Cannot pressure.

**STEP 4: HOSPICE ADMISSION**

Hospice accepts referral, completes intake, develops plan of care. Facility coordinates discharge if inpatient.

### OVERCOMING COMMON BARRIERS

**BARRIER: "Family not ready to give up treatment"**

Reframe hospice as SUPPORTIVE care. Emphasize comfort, dignity, quality of life. "Hospice helps families have more meaningful time."

**BARRIER: "Physician hesitant to certify"**

Educate physician on criteria. Provide clinical support. Position as good medicine. "You're the only one who can make this determination."

**BARRIER: "Insurance/financial confusion"**

Clarify: Medicare covers 100% of hospice. No patient cost. Facility doesn't lose billing for non-hospice care.

### COMPLIANCE & LEGAL GUARDRAILS



& Documentation must show medical necessity (physician certification, prognosis)

& NO pressure to choose one hospice over another

& Proper informed consent (patient understands choice implications)



& Referral source documented (who initiated the conversation?)

